Tumours of the urinary system

Choose the correct answer:

Urinary system https://youtu.be/I8cfNdVRkO0

Choose the correct answer:

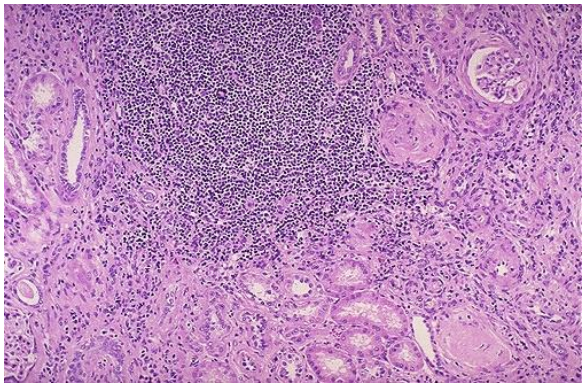
1. **Oliguria, hypertension and edema are features of**
   - **A.** Nephrotic syndrome.
   - **B.** Acute renal failure.
   - **C.** Chronic pyelonephritis.
   - D. Chronic renal failure.
   - E. Nephritic syndrome.

2. **Hypoproteinemia, heavy proteinuria and edema are features of**
   - A. Nephrotic syndrome.
   - B. Acute renal failure.
   - C. Chronic pyelonephritis.
   - D. Chronic renal failure.
   - E. Nephritic syndrome.

3. **Edema in nephrotic syndrome is caused by**
   - **A.** Increased hydrostatic pressure of blood.
   - **B.** Proteinuria and ↑ osmotic pressure of blood.
   - **C.** Hypoproteinemia and ↓osmotic pressure of blood.
   - **D.** Acute renal failure and oliguria.
   - **E.** Chronic renal failure and ↓ glomerular filtration.

4. **…….. predisposing factor for chronic pyelonephritis.**
   - **A.** Hypertension.
   - **B.** Diabetes Mellitus.
   - **C.** Contracted kidney.
   - D. Obesity.
   - E. Atherosclerosis.

5. **Chronic pyelonephritis is caused by obstruction of urinary tract or…….**
   - **A.** Stone in the bladder.
   - **B.** Stone in the ureter.
   - **C.** A tumor in the urinary bladder.
   - D. Vesico-ureteral reflux.
   - E. A tumor in the ureter.



6. **Chronic pyelonephritis is characterized histologically by:**
   A. Crescent formation and hyalinized glomeruli.
   B. Acute inflammatory cell infiltration and necrosis.
   C. Cystically dilated calyces and atrophic renal cortex.
   D. Diffuse interstitial chronic inflammatory cell infiltration.
   E. Invasion of the upper pole by polyhedral clear cells.

7. **Bacteria reach the kidney in chronic pyelonephritis through ……**
   A. Lymphatic spread from the colon.
   B. Direct spread from the uterus.
   C. Ascending from the bladder.
   D. Ascending from the small intestine.
   E. Lymphatic spread from the liver.

8. **The patient with renal stone in the urinary bladder presented with…….**
   A. Dysuria, hematuria and supra pubic pain.
   B. Polyurea, proteinuria and loin pain.
   C. Polyurea, proteinuria and generalized edema.
   D. Oliguria, hematuria and generalized edema.
   E. Oliguria, proteinuria and hypertension.

9. **There are ……types of renal stones.**
   A. Three. B. Seven. C. Five. D. Two. E. Four.
   ……..



10. Struvite stone is composed of…….

A. Magnesium-ammonium-calcium phosphate.
B. Oxalate-ammonium-calcium phosphate.
C. Cysteine- magnesium-calcium phosphate.
D. Magnesium-ammonium- potassium oxalate.
E. Magnesium-ammonium-calcium oxalate.

11…….is the most common type of stones.

A. Potassium. B. Calcium. C. Cysteine. D. Oxalate. E. Uric acid.

……..



12. Hydronephrosis means ……

A. An enlarged kidney filled with pus.
B. An enlarged kidney filled with blood.
C. An enlarged kidney filled with urine.

D. An enlarged kidney with a tumor.

E. A congenital polycystic kidney.

……



13. Stag horn stone is characterized by:

A. Multiple small brown in colour.

B. Small brown with sharp surface.

C. Green small multiple.

D. Large, irregular surface.

E. Oval, white, smooth surface.

14. Renal cell carcinoma had all these criteria except:

A. Golden yellow in colour.

B. The cross section showed areas of hemorrhage and necrosis

C. Occupied the upper or lower pole.

D. Had well-defined margins with a fibrous capsule.

E. Is called a conventional RCC.

15. Renal cell carcinoma is common in:

A. Males at 20-40Y.

B. Males at 40-60Y.

C. Males at 10-30 Y.

D. Females at 20-40 Y.

E. Children less than 4 Y.

16. Patients with renal cell carcinoma may present with:

A. Canon ball lung metastasis.

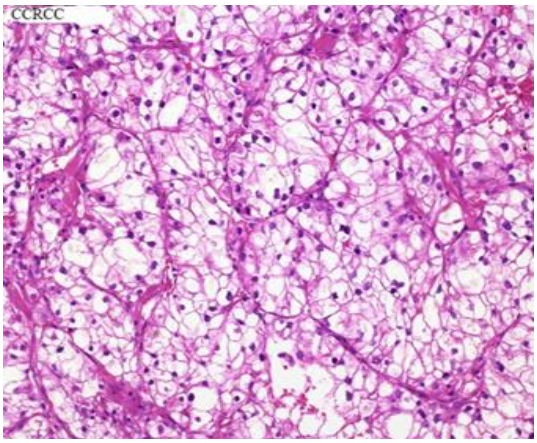
B. Liver metastasis.

C. Hypertension.

D. Gross proteinuria.

E. Acute renal failure.

……..



17. Histopathology of conventional renal cell carcinoma is characterized by:

A. Nests of polyhedral cells with clear cytoplasm and round nuclei.
B. Branching papillary processes with foamy histiocytes.
C. Spindle cell with marked nuclear pleomorphism and necrosis.
D. Sheets of neoplastic cells with round nuclei surrounded by clear halo.
E. Nests of columnar cells with deep pink cytoplasm and small nuclei.

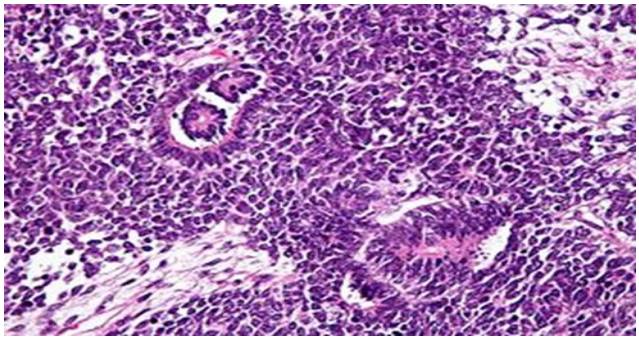
18. Wilms tumor is:

A. Renal cell carcinoma.
B. Nephroblastoma.
C. Sarcomatoid RCC.
D. Chromophobe RCC.
E. Papillary RCC.

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19. A 3-years-old child with his mother complained that she noticed a large swelling in his abdomen for 2 weeks. She recognized that the mass gradually increased in size with loss of weight and generalized weakness of her child. On physical examination the child was underweight; pale with a large soft tissue mass occupied the center of the abdomen. Sonography revealed a renal mass, 7x8cm, firm, heterogeneous in consistency. What is the diagnosis?

A. Renal cell carcinoma.
B. Hydronephrosis.
C. Chronic pyelonephritis.
D. Nephroblastoma.
E. Renal stone.

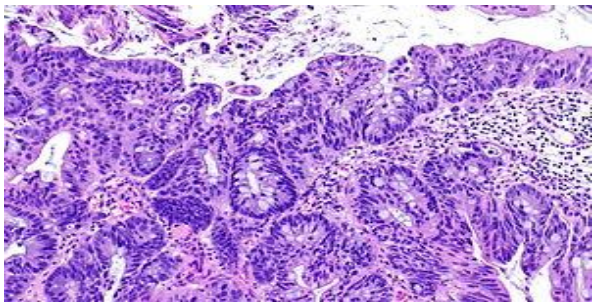


20. A biopsy from the mass showed:

A. Branching papillary processes with central fibrous core covered by neoplastic cells.
B. Neoplastic spindle cells with hyperchromatic nuclei and eosinophilic cytoplasm arranged in bundles with frequent mitosis and necrosis.
C. Blastemal cells, mesenchymal spindle cells and aborted glomeruli.
D. Interstitial chronic inflammatory cell infiltration with endarteritis obliterans and renal tubular pink casts.
E. Nests of polyhedral cells with round small nuclei and abundant clear cytoplasm.

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21. A 76-years-old obese female complained of dysuria and suprapubic pain for 2 months. She had a past history of taking anti-bilharzial drugs. Sonography showed a small urinary bladder mass 4x5cm, with necrotic areas and para-aortic LN enlargement. Transurethral resection (TUR) biopsy showed numerous glandular structures, invading the muscle wall of the bladder. What is the diagnosis?



A. Transitional cell carcinoma.
B. Squamous cell carcinoma.
C. Bilharzial cystitis.
D. Urothelial carcinoma.
E. Adenocarcinoma.

22. What is the risk factor for this lesion in the patient?

A. Old age.
B. Old bilharzial infestation.
C. Obesity.
D. Drugs.
E. Gender (female).

……

23. A 56-years-old man suffered from dysuear and hematuria for 3 months. Urine analysis showed RBCs, neutrophils and necrotic tissue. Transurethral resection (TUR) biopsy revealed cells nests and solid sheets of neoplastic

epithelial cells invading the submucosa of the bladder with areas of ulceration. What is the diagnosis?

A. Adenocarcinoma.
B. Transitional cell carcinoma.
C. Bilharzial cystitis.
D. Squamous cell carcinoma.
E. Ulcerative cystitis.

24. What are the complications of this lesion for the patient?

A. Infection.
B. Lung metastasis.
C. Fistula.
D. Spread to the rectum.
E. All of the above.

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25. A 78-years-old man suffered from chronic cough and hemoptysis (blood with the sputum) for 3weeks. Clinical examination revealed a large renal mass in the upper pole of the Lt Kidney about 6x8cm, round. A biopsy from the renal mass revealed a spindle cell proliferation with manty mitosis and necrosis. What is the diagnosis?

A. Renal stone.
B. Transitional cell carcinoma.
C. Squamous cell carcinoma.
D. Sarcomatoid RCC.
E. Oncocytoma.

26. In this patient, the lung lesion is called.

A. Hearing bone.
B. Canon ball.
C. Brun's nest.
D. Stag horn.
E. Cell nests.

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27. Thyroidization.
28. Struvite stone.
29. Bilateral hydronephrosis.
30. Nephrotic syndrome.

A. Urethral stricture.
B. Minimal changes GN.
C. Chronic pyelonephritis
D Membranous GN.
E. Triple phosphate.

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